

NOTICE INVITING TENDER (NIT)
DISTRICT HOSPITAL INFRASTRUCTURE UPGRADATION
Tender Reference No.: NHM/INFRA/MH/2025-26/089
Date of Issue: 22 January 2026

NATIONAL HEALTH MISSION – MAHARASHTRA
Health Infrastructure Division
4th Floor, Arogya Bhavan, St. George Hospital Campus
P. D'Mello Road, Mumbai – 400001
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SECTION 1: GENERAL INFORMATION

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1.1 NAME OF WORK

Civil Construction, Electrical, HVAC, Medical Gas Pipeline System (MGPS),
Biomedical Equipment Supply & Installation, IT Infrastructure, and 5-Year
Facility Management Services for Upgradation of 8 District Hospitals across
Aurangabad, Nashik, Amravati, and Latur Divisions under Pradhan Mantri
Swasthya Suraksha Yojana (PMSSY) Phase-IV, Maharashtra.

1.2 ESTIMATED PROJECT COST

Rs. 67,40,00,000/- (Rupees Sixty-Seven Crore Forty Lakhs Only)

1.3 EARNEST MONEY DEPOSIT (EMD)

Rs. 1,34,80,000/- (Rupees One Crore Thirty-Four Lakhs Eighty Thousand Only)

Via Bank Guarantee / DD in favour of Mission Director, NHM Maharashtra.

Valid for 210 days from bid submission date.

1.4 PERFORMANCE SECURITY

5% of Contract Value – Bank Guarantee valid for contract period + 90 days.

1.5 DEFECT LIABILITY PERIOD

36 months from date of Completion Certificate (civil and MEP works).

Biomedical equipment warranty: 5 years comprehensive (parts + labour).

1.6 TENDER DOCUMENT FEE

Rs. 50,000/- (Non-refundable) via RTGS to A/C No. 1122334455,

IFSC: MAHB0001234, Bank of Maharashtra, Fort Branch, Mumbai.

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SECTION 2: SCHEDULE OF KEY DATES

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2.1 DATE OF PUBLICATION : 22 January 2026

2.2 MANDATORY SITE VISITS (ALL 8 HOSPITALS): 30-31 January 2026

2.3 PRE-BID CONFERENCE : 05 February 2026 at 11:00 AM

2.4 LAST DATE FOR SUBMISSION OF QUERIES : 10 February 2026

2.5 RESPONSE TO QUERIES / CORRIGENDUM : 15 February 2026

2.6 LAST DATE FOR BID SUBMISSION : 05 March 2026 up to 5:00 PM

2.7 TECHNICAL BID OPENING : 06 March 2026 at 11:00 AM

2.8 FINANCIAL BID OPENING : To be notified post technical evaluation

2.9 WORK COMPLETION PERIOD : 18 months from date of work order

2.10 FACILITY MANAGEMENT PERIOD : 5 years post-commissioning

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SECTION 3: ELIGIBILITY CRITERIA

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3.1 LEGAL ENTITY

Company/firm registered under Companies Act 2013 or LLP. Valid GST, PAN, and Contractor registration with Maharashtra PWD (Class AA or equivalent).

3.2 FINANCIAL ELIGIBILITY

3.2.1 Average Annual Turnover: Minimum Rs. 45 Crores per annum for last 3 financial years (FY 2022-23, FY 2023-24, FY 2024-25).

3.2.2 Net Worth: Positive net worth as per latest audited balance sheet.

3.2.3 Working Capital / Credit Facility: Minimum Rs. 12 Crores (solvency certificate from scheduled commercial bank required).

3.3 TECHNICAL ELIGIBILITY – SIMILAR WORK EXPERIENCE

The bidder must have successfully completed at least:

ONE hospital / healthcare infrastructure project of value not less than Rs. 25 Crore OR TWO such projects each not less than Rs. 15 Crore during the last 7 years.

"Similar Work" includes: Hospital construction/upgradation, medical college

infrastructure, multi-specialty clinic, or government health centre with civil + MEP + biomedical equipment scope under a single contract.

3.4 SPECIALIST SUB-CONTRACTOR / VENDOR EMPANELMENT

Bidder shall provide authorisation / empanelment certificates from:

- (a) Medical Gas Pipeline System (MGPS) specialist (HTMS/BeaconMedaes/equivalent)
- (b) Biomedical equipment OEMs (GE Healthcare / Philips / Siemens / equivalent)
- (c) HVAC system installer empanelled with ISHRAE
- (d) Fire NOC consultant registered with Maharashtra Fire Services

3.5 KEY PERSONNEL REQUIREMENTS

3.5.1 Project Director: B.E./B.Tech Civil, PG preferred, 15 years experience with at least 3 hospital projects.

3.5.2 Civil Site Engineer (per hospital): B.E. Civil, 7 years experience.

3.5.3 MEP Engineer: B.E. Electrical/Mechanical, 8 years experience in hospital MEP including MGPS.

3.5.4 Biomedical Engineer: B.E. Biomedical/Electronics, 5 years experience in hospital equipment installation and maintenance.

3.5.5 Facility Manager: MBA/B.E. with 6 years hospital facility management.

3.5.6 Infection Control Consultant: Registered nurse/medical professional with NABH-aligned infection control certification.

3.6 CERTIFICATIONS REQUIRED

ISO 9001:2015 (Quality Management) mandatory.

NABH-compliant construction methodology preferred (20 additional marks).

3.7 CONSORTIUM

Maximum 2-firm consortium. Lead member: minimum 55% stake and must

individually satisfy similar work and financial criteria.

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SECTION 4: SCOPE OF WORK

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4.1 CIVIL WORKS (ALL 8 HOSPITALS)

- 4.1.1 Demolition of dilapidated structures and debris disposal
- 4.1.2 New OPD block construction (G+2 floors, 800 sqm per hospital)
- 4.1.3 ICU/HDU ward upgradation (20 beds per hospital, modular design)
- 4.1.4 Operation Theatre (OT) complex: 2 modular OTs per hospital
(laminar airflow, HEPA filtration, positive pressure)
- 4.1.5 Labour room and SNCU (Sick Newborn Care Unit) upgradation
- 4.1.6 Pharmacy, laboratory, and blood bank infrastructure
- 4.1.7 Mortuary upgradation with cold storage
- 4.1.8 Administrative block, canteen, and waiting areas
- 4.1.9 Ramps, lifts (2 per hospital), and accessibility compliance
as per Rights of Persons with Disabilities Act 2016
- 4.1.10 External development: parking, drainage, waste management area,
boundary wall, security cabin, generator yard

4.2 MECHANICAL, ELECTRICAL AND PLUMBING (MEP)

- 4.2.1 High Tension (HT) power supply connection and substation (500 KVA)
- 4.2.2 DG Set (500 KVA, with AMF panel) for 100% critical area backup
- 4.2.3 Solar rooftop (200 KWp per hospital) – 1.6 MWp aggregate
- 4.2.4 Electrical distribution: LT panels, UPS (online double conversion,

30-minute backup for ICU/OT), wiring, fixtures

4.2.5 Medical Gas Pipeline System (MGPS):

- Oxygen: manifold, copper pipeline, bedhead units (ICU + OT)
- Nitrous Oxide, Medical Air, Vacuum (OT complex)
- Alarm panels, zone valve boxes, pressure gauges

4.2.6 HVAC: central AC for OT (Class 100 laminar flow), ICU, CSSD;

split/cassette units for OPD, wards, and admin areas

4.2.7 Firefighting: sprinklers, hydrants, fire alarm (addressable),

fire NOC compliance

4.2.8 Plumbing: CPVC hot/cold water, ETP (30 KLD capacity per hospital),

STP (10 KLD), rainwater harvesting

4.2.9 Nurse call system and patient entertainment system (ICU/wards)

4.3 BIOMEDICAL EQUIPMENT SUPPLY AND INSTALLATION

4.3.1 Digital X-Ray System (DR) with PACS connectivity – 1 per hospital

4.3.2 Ultrasound Machine (B&W + Colour Doppler) – 2 per hospital

4.3.3 Anaesthesia Workstation with ventilator – 2 per hospital (OT)

4.3.4 Multi-parameter Patient Monitor (ICU grade, 12-lead ECG) – 20 per hospital

4.3.5 Infant Radiant Warmer and Phototherapy Unit – 4 sets per hospital (SNCU)

4.3.6 Defibrillator (AED + Manual, biphasic) – 3 per hospital

4.3.7 Autoclave (134-litre, Class B) for CSSD – 2 per hospital

4.3.8 Biochemistry Analyser (fully automated, 400 tests/hour) – 1 per hospital

4.3.9 Haematology Analyser (5-part differential) – 1 per hospital

4.3.10 Laparoscopy Tower (Full HD, 10 mm 0°/30°, insufflator) – 1 per hospital

4.3.11 C-Arm Fluoroscopy Unit (mobile, 9-inch II) – 1 per hospital

4.3.12 Dental Chair and Equipment Set – 1 per hospital

4.4 IT AND DIGITAL INFRASTRUCTURE

- 4.4.1 Hospital Management Information System (HMIS) – cloud-based, ABDM (Ayushman Bharat Digital Mission) compliant
- 4.4.2 Electronic Medical Records (EMR) with HL7/FHIR integration
- 4.4.3 Structured cabling (Cat 6A), Wi-Fi (802.11ax) coverage, server room
- 4.4.4 CCTV surveillance (IP cameras, 30-day NVR storage) – 50 cameras/hospital
- 4.4.5 Biometric access control for OT, ICU, pharmacy, blood bank
- 4.4.6 Telemedicine endpoint (HD video, medical peripherals) – 1 per hospital

4.5 TESTING, COMMISSIONING AND HANDOVER

- 4.5.1 Air change rate validation for OT and ICU (NABH standards)
- 4.5.2 MGPS pressure and flow testing (HTM 02-01 / IS 7285)
- 4.5.3 Electrical load testing and power quality measurement
- 4.5.4 Biomedical equipment acceptance testing (FAT + SAT)
- 4.5.5 NABH pre-accreditation gap assessment and remediation
- 4.5.6 Training: 10-day structured training for clinical and technical staff at each hospital (minimum 30 staff per hospital)

4.6 FACILITY MANAGEMENT SERVICES (5 YEARS)

- 4.6.1 Civil maintenance: minor repairs, painting cycle, waterproofing
- 4.6.2 MEP maintenance: PPM schedule for electrical, HVAC, MGPS, lifts
- 4.6.3 Biomedical equipment maintenance: AMC with 98% uptime guarantee
- 4.6.4 Housekeeping: NABH-compliant colour-coded cleaning protocols
- 4.6.5 Biomedical waste management: BMW handling per BMW Rules 2016
- 4.6.6 Help desk and CAFM (Computer-Aided Facility Management) software
- 4.6.7 Annual energy audit and water audit reports
- 4.6.8 Security services: 3 guards per hospital per shift, 24x7

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SECTION 5: BILL OF QUANTITIES (BOQ)

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SCHEDULE A – CIVIL WORKS (PER HOSPITAL × 8)

S.No	Description	Unit	Qty	Rate(Rs.)	Amount(Rs.)
A.1	Demolition and debris disposal	Sqm	3200	850	2720000
A.2	OPD Block construction (G+2, RCC framed)	Sqm	6400	28000	179200000
A.3	ICU/HDU ward upgradation (20 beds)	Sqm	3200	32000	102400000
A.4	Modular OT complex (2 OTs, laminar airflow)	Sqm	1600	65000	104000000
A.5	Labour room and SNCU upgradation	Sqm	2400	30000	72000000
A.6	Pharmacy, lab, blood bank fit-out	Sqm	3200	26000	83200000
A.7	Mortuary with cold storage	Sqm	640	35000	22400000
A.8	External development (parking, drainage, wall)	LS	8	2800000	22400000
A.9	Passenger Lift (1000 kg, hospital grade)	No.	16	2200000	35200000
A.10	Ramps and accessibility works	LS	8	350000	2800000

SCHEDULE B – MEP WORKS

B.1	HT connection and 500 KVA substation	No.	8	4500000	36000000
B.2	DG Set 500 KVA with AMF panel	No.	8	3800000	30400000
B.3	Solar Rooftop 200 KWp (per hospital)	KWp	1600	58000	92800000
B.4	LT distribution, UPS (online 40 KVA), wiring	LS	8	3200000	25600000
B.5	Medical Gas Pipeline System (MGPS) complete	LS	8	5500000	44000000
B.6	HVAC – OT Class 100 laminar flow + ICU + OPD	LS	8	6200000	49600000

B.7	Firefighting + addressable fire alarm	LS	8	1800000	14400000
B.8	Plumbing + ETP (30 KLD) + STP (10 KLD)	LS	8	2400000	19200000
B.9	Nurse call system + patient entertainment	LS	8	850000	6800000

SCHEDULE C – BIOMEDICAL EQUIPMENT

C.1	Digital X-Ray DR system with PACS	No.	8	3500000	28000000
C.2	Ultrasound (Colour Doppler)	No.	16	1200000	19200000
C.3	Anaesthesia Workstation with ventilator	No.	16	1800000	28800000
C.4	Multi-parameter Patient Monitor (ICU grade)	No.	160	280000	44800000
C.5	Infant Radiant Warmer + Phototherapy (set of 4)	Set	8	620000	4960000
C.6	Defibrillator (biphasic AED + Manual)	No.	24	185000	4440000
C.7	Autoclave Class B 134-litre (set of 2)	Set	8	550000	4400000
C.8	Biochemistry Analyser (fully automated)	No.	8	2200000	17600000
C.9	Haematology Analyser (5-part)	No.	8	950000	7600000
C.10	Laparoscopy Tower (Full HD)	No.	8	2800000	22400000
C.11	C-Arm Fluoroscopy (mobile 9-inch)	No.	8	3200000	25600000
C.12	Dental Chair and Equipment Set	Set	8	480000	3840000

SCHEDULE D – IT AND DIGITAL INFRASTRUCTURE

D.1	HMIS (cloud, ABDM compliant, 5-yr SaaS)	LS	1	8500000	8500000
D.2	Structured cabling Cat 6A + Wi-Fi 6 (per hosp)	LS	8	1200000	9600000
D.3	CCTV (50 IP cameras + NVR, per hospital)	LS	8	850000	6800000
D.4	Biometric access control (OT, ICU, pharmacy)	LS	8	380000	3040000
D.5	Telemedicine endpoint (per hospital)	No.	8	420000	3360000
D.6	Server room setup (rack, UPS, cooling)	LS	8	650000	5200000

SCHEDULE E – FACILITY MANAGEMENT (5 YEARS × 8 HOSPITALS)

E.1 FMS Year 1 (all 8 hospitals)	LS	1	12000000	12000000
E.2 FMS Year 2	LS	1	12600000	12600000
E.3 FMS Year 3	LS	1	13230000	13230000
E.4 FMS Year 4	LS	1	13890000	13890000
E.5 FMS Year 5	LS	1	14590000	14590000

TOTAL ESTIMATED COST (A+B+C+D+E): Rs. 67,40,00,000/-

NOTE: Rates inclusive of GST, freight, insurance, installation, and commissioning.

Biomedical equipment must be from approved CMSS / GEM-listed OEMs wherever applicable.

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SECTION 6: COMMERCIAL TERMS AND CONDITIONS

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6.1 PAYMENT TERMS

Stage 1: 5% on signing of Contract Agreement (mobilisation advance against BG)

Stage 2: 40% on pro-rata basis against certified civil work progress (RA bills,
certified by NHM project engineer and PMC)

Stage 3: 20% against MEP works completion certification per hospital

Stage 4: 15% against biomedical equipment supply, installation, and
commissioning with acceptance test certificate

Stage 5: 10% on issuance of Completion Certificate and NABH gap report

Stage 6: 5% on achieving NABH Entry Level Accreditation for all 8 hospitals

Stage 7: Remaining 5% as retention money; released after 36-month DLP expiry

FMS Payment: Monthly in arrears. Subject to:

- CAFM-generated performance report showing uptime compliance
- NHM satisfaction certificate from Hospital Medical Superintendent
- BMW disposal manifests and housekeeping audit score $\geq 80\%$

6.2 LIQUIDATED DAMAGES

6.2.1 Delay in Completion: 0.5% per week delay, maximum 10% of contract value.

6.2.2 NABH Accreditation Failure: Rs. 5,00,000/- per hospital per month of delay beyond 6 months post-completion.

6.2.3 Biomedical Equipment Downtime: Rs. 8,000/- per equipment per day beyond agreed MTTR (48 hours for critical equipment).

6.2.4 FMS SLA Breach: Proportionate monthly deduction from FMS invoice if housekeeping score $< 80\%$ or biomedical uptime $< 98\%$.

6.3 PERFORMANCE GUARANTEE

Bank Guarantee: 5% of contract value.

Valid for 18 months (construction) + 5 years (FMS) + 90 days.

Separate BG required for FMS period, submitted 30 days before FMS commencement.

6.4 PRICE VARIATION

No price variation on supply, civil, or MEP works.

FMS: 5% per annum escalation from Year 2 (pre-built into Schedule E).

6.5 INSURANCE

Contractor's All Risk (CAR): full contract value.

Professional Indemnity: Rs. 5 Crore per occurrence (for biomedical and MGPS).

Workmen Compensation: as per Employees' Compensation Act.

Third Party Liability: Rs. 3 Crore per occurrence.

Patient Safety Liability: Rs. 1 Crore per incident during FMS period.

6.6 MOBILISATION ADVANCE

15% of contract value available as mobilisation advance against unconditional Bank Guarantee of equivalent value. Recovered in equal installments from RA bills starting from 3rd bill.

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SECTION 7: RISK ASSESSMENT

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7.1 HIGH RISK ITEMS

7.1.1 Infection Control Risk: Construction in active hospital premises.

Contractor must implement ICRA (Infection Control Risk Assessment) methodology. Any healthcare-associated infection (HAI) outbreak attributed to construction negligence shall be contractor's liability.

7.1.2 MGPS Safety Risk: Medical gas leakage in ICU/OT environment is

life-threatening. All MGPS works must be executed by certified specialist with HTM 02-01 compliance. Independent third-party pressure testing mandatory before commissioning.

7.1.3 NABH Compliance Risk: Failure to achieve NABH Entry Level

Accreditation within agreed timeline will trigger LDs. NHM may appoint NABH consultants at contractor's cost after 3-month delay.

7.1.4 Biomedical Equipment Regulatory Risk: All equipment must have CDSCO registration (Medical Devices Rules 2017). Supply of non-registered equipment shall result in rejection and replacement at contractor's cost with no time extension.

7.2 MEDIUM RISK ITEMS

7.2.1 Utility Shifting Risk: Existing water, sewage, and electrical lines within hospital campus may obstruct new construction. Utility shifting by contractor at own cost; NHM to provide existing drawings (accuracy not guaranteed).

7.2.2 Hospital Continuity Risk: Hospitals to remain operational during construction. Contractor to submit Phased Construction Plan ensuring no disruption to emergency and OPD services.

7.2.3 Seasonal Risk: Maharashtra monsoon (June–September) may affect civil work progress. Contractor to factor weather allowance into program; no EOT for normal monsoon delays.

7.3 LOW RISK ITEMS

7.3.1 Equipment Lead Time Risk: Imported biomedical equipment (C-Arm, Biochemistry Analyser) may have 16–20 week lead time. Contractor to place orders within 30 days of work order.

7.3.2 Staff Training Risk: Clinical staff may resist new HMIS workflows.

Contractor to provide go-live support for 3 months post-training.

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SECTION 8: EVALUATION CRITERIA

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8.1 TWO-BID SYSTEM (Technical + Financial)

8.2 TECHNICAL EVALUATION (Pass/Fail + Scoring)

Criterion	Max Marks

Similar Work Experience (hospital projects)	25
Financial Capacity	10
Key Personnel Qualifications	20
Technical Methodology and Phased Work Program	15
Biomedical OEM Partnerships	10
NABH Experience / Certifications	10
Local Presence and Service Infrastructure	10
TOTAL	100

Minimum Qualifying Score: 70 out of 100

Bonus: NABH-compliant methodology certification: +5 marks (beyond 100)

8.3 FINANCIAL EVALUATION

Lowest Valid Financial Bid (L1) basis.

NHM reserves right to negotiate with L1.

8.4 NEGOTIATIONS

NHM may seek up to 5% reduction from L1 bidder.

Refusal to negotiate: L2 bidder invited at L1-negotiated price.

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SECTION 9: GENERAL CONDITIONS OF CONTRACT

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9.1 GOVERNING LAW

Governed by laws of India. Jurisdiction: High Court, Bombay.

9.2 DISPUTE RESOLUTION

Step 1: Departmental review committee (30 days).

Step 2: Arbitration under Arbitration and Conciliation Act, 1996.

Three-member panel: one each nominated by NHM and Contractor; third appointed by mutual agreement or by Principal Secretary, Health, Maharashtra.

Venue: Mumbai. Language: English/Marathi.

9.3 FORCE MAJEURE

Natural disaster, epidemic/pandemic declared by GoI/GoM, earthquake, unprecedented flood (beyond 100-year return period), act of war.

Notice within 7 days. FMS SLA suspended during declared Force Majeure.

9.4 TERMINATION FOR CONVENIENCE

NHM may terminate on 90-day written notice. Contractor entitled to

payment for completed and accepted work and reasonable demobilisation costs.

9.5 TERMINATION FOR CAUSE

Immediate termination for:

- (a) Abandonment of work at 3 or more hospitals simultaneously
- (b) Supply of non-CDSCO registered biomedical equipment
- (c) Fraudulent FMS billing or falsified CAFM reports
- (d) Patient safety incident caused by contractor negligence (ICU/OT)
- (e) Contractor becoming insolvent or receivership

9.6 SUBCONTRACTING

Up to 35% of contract value permitted with prior written approval.

MGPS and biomedical equipment works may only be subcontracted to empanelled specialist firms. Contractor remains fully responsible.

9.7 CHANGE ORDERS

NHM may add up to 2 additional hospitals at BOQ rates within 12 months of work order. May reduce scope by up to 15% without compensation. Changes beyond these limits require renegotiation.

9.8 INTELLECTUAL PROPERTY

HMIS, CAFM software, and custom integrations developed for this project shall vest with NHM Maharashtra upon final payment.

OEM firmware, proprietary biomedical software retains vendor IP.

9.9 CONFIDENTIALITY

Patient data, hospital occupancy, clinical protocols, and financial data accessed during FMS are strictly confidential under IT Act 2000

and DPDP Act 2023. Data breach liability: up to Rs. 1 Crore per incident.

9.10 ANTI-CORRUPTION AND INTEGRITY PACT

Contractor to sign NHM Integrity Pact. Corrupt practice results in termination, forfeiture of all bank guarantees, and debarment from NHM Maharashtra for 5 years.

9.11 SAFETY AND LABOUR LAWS

Full compliance with Building and Other Construction Workers Act 1996, Factories Act, ESIC, PF. Safety officer mandatory on each site.

Any fatality: immediate stoppage and NHM investigation committee.

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SECTION 10: DOCUMENTS REQUIRED WITH BID

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ENVELOPE 1 – EMD AND FEES

- (a) EMD Instrument (Bank Guarantee / DD)
- (b) Tender Document Fee receipt
- (c) Mandatory site visit attendance certificate (all 8 hospitals)

ENVELOPE 2 – TECHNICAL BID

- (a) Form of Tender (signed by authorised signatory with company seal)
- (b) Certificate of Incorporation / Registration
- (c) GST Certificate, PAN, Maharashtra PWD Contractor Registration
- (d) ISO 9001:2015 Certificate

- (e) Audited Financial Statements – last 3 years with CA certificate
- (f) Banker's Solvency Certificate (not older than 3 months)
- (g) Completion Certificates for similar hospital works (attested by notary)
- (h) MGPS Specialist empanelment / authorisation certificate
- (i) Biomedical OEM authorisation letters (GE / Philips / Siemens or equivalent)
- (j) NABH consultant / NABH experience certificates (if applicable)
- (k) CVs with supporting documents for all key personnel
- (l) Detailed Technical Methodology with phased construction program
- (m) ICRA methodology document for hospital infection control during construction
- (n) List of ongoing and completed hospital projects with references
- (o) Self-declaration of non-blacklisting
- (p) Power of Attorney for authorised signatory
- (q) Integrity Pact (duly signed)

ENVELOPE 3 – FINANCIAL BID

- (a) Completed BOQ (Schedules A to E) with unit rates
- (b) Summary price sheet with total bid value
- (c) Covering letter confirming bid validity of 180 days

All envelopes sealed separately, placed in outer envelope superscribed:

"NIT No. NHM/INFRA/MH/2025-26/089 – District Hospital Upgradation –

Do Not Open Before 06 March 2026"

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END OF TENDER DOCUMENT

National Health Mission – Maharashtra | Mumbai

Tender Ref: NHM/INFRA/MH/2025-26/089

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